NOVOGENIA · ACADEMY

# FREQUENTLY ASKED QUESTIONS PERSONALIZED SUPPLEMENTATION

Answers to typical client questions about personalized supplementation and microtransporter technology.

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## 01 · BASICS — WHY PERSONALIZED?

### Why do I need personalized supplementation at all?

Because your body genetically utilizes some nutrients better or worse than another person — and because your lifestyle (exercise, sleep, stress, nutrition) additionally shapes your individual need. Standard pills assume the "average person" and therefore cover many individual needs either too little or too much.

### What is the difference from standard multivitamins from the drugstore?

Three main points: (1) The selection of nutrients and their dosage is tailored to your genes; (2) the release is timed (e.g., Slow-Release Vitamin C) to keep blood levels constant; (3) competing nutrients like Calcium and Zinc are released spatially and temporally separated so they don't block each other.

### How many genes does the analysis consider?

Over 60 nutrient-relevant gene variants — including vitamin-metabolism genes (folate-MTHFR, Vit-D-VDR, Vit-B12-FUT2), mineral-absorption genes (iron-HFE, calcium-LCT), and antioxidant defense systems (GST, SOD, CAT).

## 02 · MICROTRANSPORTER TECHNOLOGY

### What are microtransporters?

Microtransporters are tiny pellets in the millimetre range, each carrying individual nutrients in specific protective shells. Each microtransporter "releases" at a different time and a different location in the GI tract — so even normally competing nutrients can be taken together but absorbed separately.

### Why is Vitamin C released as slow-release?

Vitamin C has a very short biological half-life (~30 minutes). A single high dose is largely excreted unused. Slow-Release in the microtransporters mimics the natural, continuous

intake from food across the day and keeps blood levels stable — physiologically much more sensible.

### How does it work with Calcium and Zinc that block each other?

Calcium and Zinc compete for the same intestinal transporters — taking both together yields less of both. In the microtransporter technology, Zinc is released in the upper small intestine immediately, Calcium hours later in the deeper intestinal segment. Both are taken at once, but released at different places and times — no competition.

## 03 · PRACTICAL QUESTIONS

### Do I have to take the pills at fixed times?

We recommend a consistent time of day (e.g. with breakfast) to keep blood levels stable. You don't have to watch the minute — a rough routine is enough. The most important thing is taking them regularly at all.

### How fast will I notice an effect?

It varies by nutrient. Some effects (e.g. better energy with B-vitamins) appear within days to weeks; others (e.g. bone metabolism with Vitamin D) develop over months. Personalized supplementation is not a fast-acting medication — it corrects long-term nutrient gaps.

### Do I need supplementation forever?

For genetically caused metabolic weaknesses yes, because the predisposition lasts a lifetime. Lifestyle-driven needs (e.g. higher need during stress, exercise, illness) can change. The analysis also shows you where your genetics produce a permanently elevated need and where lifestyle adjustment alone is sufficient.

### Can I combine supplementation with medications?

Generally yes — all nutrients are dosed in food-typical amounts. With chronic disease or long-term medication you should discuss this with your physician, because individual nutrients (e.g. Vitamin K, iron) can interact with certain drugs.

### How does personalized cost compare to drugstore multivitamins?

Significantly more per day — typically 3-5× the cheapest drugstore products. In return you get a genetically tailored composition that probably actually gets absorbed instead of being largely excreted unused. The "per mg of vitamin" comparison is misleading; "per mg that actually reaches the blood" is more meaningful.